Causality Assessment Report

Pharmacovigilance & Drug Safety Analysis

Task ID: task3
Subject: ADR Evaluation

1. CASE OVERVIEW & PATIENT PROFILE

Patient Demographic	45-year-old Female	**Suspected Drug**	Amoxicillin
Adverse Event Reported	Skin Rash	**Onset Latency**	3 days post-initiation
Dechallenge Outcome	Positive (Symptom resolution following drug discontinuation)		

2. CAUSALITY ANALYSIS CRITERIA

Evaluation Parameter	Clinical Findings / Observation	Assessment
Temporal Relationship	Skin rash developed 3 days following the commencement of oral Amoxicillin. This interval matches typical delayed-type hypersensitivity.	Compatible / Plausible
Dechallenge	Condition markedly improved and fully resolved after withdrawal of Amoxicillin therapy without recurrence.	Positive Dechallenge
Rechallenge	Rechallenge was not attempted due to clinical risk and ethical considerations for patient safety.	Not Applicable / Unperformed
Alternative Etiologies	No confounding medications or viral exanthem patterns documented that fully account for the rapid onset and resolution.	Unlikely / Ruled Out

3. NARANJO ALGORITHM EVALUATION

Standard Question	Score	Result
1. Previous conclusive reports on this reaction? (Amoxicillin & skin rash)	+1	Yes
2. Did the adverse event appear after the suspected drug was administered?	+2	Yes
3. Did the adverse reaction improve when the drug was discontinued?	+1	Yes
4. Did the adverse reaction reappear when the drug was re-administered?	0	Not Done
5. Are there alternative causes that could on their own have caused the reaction?	+2	No
Total Computed Naranjo Score	**+6**	**Probable (Score 5-8)**

4. Final Causality Conclusion

Under both the **WHO-UMC Causality Assessment Scale** and the **Naranjo Adverse Drug Reaction Probability Scale**, this case is definitively classified as **PROBABLE / LIKELY**.

- **Reasoning:** Event onset presents a reasonable temporal sequence to drug administration, followed a recognized response pattern to beta-lactam antibiotics, confirmed by a clinically positive dechallenge, and cannot be reasonably explained by concurrent disease or alternative therapies.
- **Regulatory Recommendation:** Submit individual case safety report (ICSR) to the national pharmacovigilance center and counsel the patient regarding documented penicillin-class hypersensitivity.